

Martin, Elizabeth

MRN: 110704351

Office Visit 12/4/2024
PAV WH Gynecology

Provider: Fredericks, Tricia I, MD (Gynecologic Oncology)
Primary diagnosis: Endometrial cancer (CMS/HCC)
Reason for Visit: Follow-up

Progress Notes

Fredericks, Tricia I, MD (Physician) • Gynecologic Oncology

Patient ID:

Elizabeth Martin is a 69 y.o. female.

Referring Physician:

No referring provider defined for this encounter.

Primary Care Provider:

Giles, Charles S, MD

History of Present Illness:

Chief Complaint: Serous endometrial cancer

Elizabeth Martin is a 69 y.o. female seen in consultation for a newly diagnosed Stage IIIb serous endometrial cancer, s/p Robotic TLH/BSO/SLN/biopsy of sigmoid nodules(+ but completely resected per op report), washings+. She initially had spotting. She went to her PCP then a GYN for US and biopsy. She had a preop CT without any evidence of metastatic disease or lymphadenopathy. She is doing well physically after surgery. Small amount of discharge but sounds like recovering well. She has a post-op appt with Dr. Cottrill this afternoon.

There was some confusion for her after surgery. She thought she would have to find an oncologist for treatment. Her PCP, Dr. Giles recommended Markey Cancer Center so she was referred here. She does like UK and has decided she would like treatment here. She likes how quickly she was able to get an appointment. She has questions regarding the pathology and treatment recommendations.

S/p carbo/taxol x6 followed by VBT.

Radiation Treatment History:

Treatment Site: HDR Vaginal Cuff Brachytherapy using 3.0 cm diameter cylinder and delivered to a depth of 0.5 cm from the vaginal mucosal applicator

Total Dose: 700 cGy x 3 fractions

Date RT completed: 7/30/2024; 8/7/2024; 8/14/2024

At her post radiation appt, she was having vague symptoms in pelvis. Also with some CT changes, CA125 was elevated. She returns today to discuss her PET.

Interval History: She presents today to discuss her PET scan. She does still have some vague pain on her left side and deep in her pelvis. No other real symptoms.

PMH: HTN, HL, anxiety, obesity Body mass index is 32.7 kg/m²., melanoma

PSH: Robotic TLH/BSO/SLN/biopsies

Meds: lisinopril, lorazepam, benadryl, aleve

Aller: NKMA

SocHx:

FamHx: father - kidney CA, Mother - uterine CA surgery with Dr. DePriest

OB/GYN Hx: G2P2
Menopause: 2010
Mammogram: All normal, next in March (was rescheduled from Jan)
Colonoscopy: Normal 2021

Review of Systems
Constitutional: Negative.
HENT: Negative.
Eyes: Negative.
Respiratory: Negative.
Cardiovascular: Negative.
Gastrointestinal: Negative.
Genitourinary: Positive for vaginal bleeding.
Musculoskeletal: Positive for arthralgias.
Skin: Negative.
Neurological: Positive for numbness.
Hematological: Negative.
Psychiatric/Behavioral: The patient is nervous/anxious.

[Cancer Staging](#)
No matching staging information was found for the patient.

Treatment Details	
Treatment goal	[No plan goal]
Plan Name	PACLitaxel / CARBOplatin Every 21 Days
Status	Inactive
Start Date	1/19/2024
End Date	5/17/2024
Provider	Tricia I Fredericks, MD
Chemotherapy	CARBOplatin (Paraplatin) 414 mg in sodium chloride 0.9 % 250 mL chemo IVPB, 414 mg, Intravenous, Once, 6 of 6 cycles Administration: 414 mg (1/19/2024), 400 mg (2/9/2024), 424.5 mg (3/4/2024), 421 mg (3/29/2024), 454 mg (4/19/2024), 442 mg (5/17/2024) PACLitaxel (Taxol) 318 mg in sodium chloride 0.9 % 500 mL chemo IVPB, 175 mg/m2 = 318 mg, Intravenous, Once, 6 of 6 cycles Dose modification: 135 mg/m2 (original dose 175 mg/m2, Cycle 6, Reason: Toxicity/Complication) Administration: 318 mg (1/19/2024), 318 mg (2/9/2024), 318 mg (3/4/2024), 318 mg (3/29/2024), 318 mg (4/19/2024), 246

mg (5/17/2024)

aprepitant (Cinvanti) 130 MG/18ML IV 130 mg, 130 mg,
Intravenous, Once, 6 of 6 cycles
Administration: 130 mg (1/19/2024), 130 mg (2/9/2024), 130
mg (3/4/2024), 130 mg (3/29/2024), 130 mg (4/19/2024), 130
mg (5/17/2024)

Subjective

History:

Past Medical History:

Diagnosis	Date
- HTN (hypertension) - Hx antineoplastic chemo	January 19, 2024
- Hx of melanoma excision - Skin cancer - Uterine cancer (CMS/HCC)	January 2016 November 9, 2023

Allergies

Allergen	Reactions
Paclitaxel <i>Shortness of breath, back pain, flushing, headache.</i>	Other - please document in the comment field

Current Outpatient Medications:

- amLODIPine (Norvasc) 5 MG tablet, , Disp: , Rfl:
- citalopram (CeleXA) 10 MG tablet, , Disp: , Rfl:
- diphenhydrAMINE (Benadryl) 25 MG capsule, Take 1 capsule (25 mg) by mouth at night if needed., Disp: , Rfl:
- lisinopril 10 MG tablet, Take 1 tablet (10 mg) by mouth 1 (one) time each day., Disp: , Rfl:
- LORazepam (Ativan) 1 MG tablet, , Disp: , Rfl:
- traMADol (Ultram) 50 MG tablet, Take 1 tablet (50 mg) by mouth every 8 (eight) hours if needed for severe pain., Disp: 30 tablet, Rfl: 0

Family History

Problem	Relation	Name	Age of Onset
Endometrial cancer	Mother	Lucy Geraldine Grant	
Cancer	Mother	Lucy Geraldine Grant	
Dementia	Mother	Lucy	

		Geraldine Grant
• Heart disease	Mother	Lucy Geraldine Grant
• Hypertension	Mother	Lucy Geraldine Grant
• Kidney cancer	Father	Herschel Grant
• Cancer	Father	Herschel Grant
• Kidney cancer	Sister	
• Stroke	Maternal Grandfather	Lester Davidson
• Stroke	Maternal Grandmother	Lura Davidson
• Cancer	Sister	Linda Kessler
• Cancer	Brother	David Grant
• Hypertension	Sister	Patsy Melton

Tobacco Use: Low Risk (11/22/2024)

- Patient History
- Smoking Tobacco Use: Never
 - Smokeless Tobacco Use: Never
 - Passive Exposure: Never

Social History

Substance and Sexual Activity

Alcohol Use Never

Social History

Substance and Sexual Activity

Drug Use Never

Social Connections: Unknown (11/16/2023)

- Received from Baptist Healthcare System, Baptist Healthcare System
- Family and Community Support
- Help with Day-to-Day Activities: Not on file
 - Lonely or Isolated: Not on file

Objective **Physical Exam:**Vital Signs for this encounter:

BSA: 1.88 meters squared

Visit Vitals

BP	(!) 147/86
Pulse	92
Temp	(!) 35.9 °C (96.6 °F)
Resp	14
Wt	81.1 kg (178 lb 12.7 oz)
LMP	12/01/2010
BMI	32.70 kg/m ²
OB Status	Hysterectomy
Smoking Status	Never
BSA	1.88 m ²

Physical Exam

Vitals and nursing note reviewed. Exam conducted with a chaperone present.

Constitutional:Appearance: Normal appearance. She is well-developed. She is **obese**.HENT:

Head: Normocephalic and atraumatic.

Right Ear: External ear normal.

Left Ear: External ear normal.

Nose: No congestion or rhinorrhea.

Mouth/Throat:

Mouth: Mucous membranes are moist.

Eyes:

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:Rate and Rhythm: **Tachycardia** present.Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: No wheezing.

Abdominal:Comments: **deferred**Genitourinary:

General: Normal vulva.

Exam position: Lithotomy position.

Musculoskeletal:

Cervical back: Normal range of motion.

Skin:

General: Skin is dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Neurological:

General: No focal deficit present.

Mental Status: She is alert. Mental status is at baseline.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

Performance Status:

Asymptomatic

Pain Scale: 0

Results:**WBC Count (10³/uL)**

Date/Time	Value
03/04/2024 0826	4.52

HGB (g/dL)

Date/Time	Value
03/04/2024 0826	13.9

HCT (%)

Date/Time	Value
03/04/2024 0826	42.2

Platelet Count (10³/uL)

Date/Time	Value
03/04/2024 0826	165

Creatinine, Plasma (mg/dL)

Date/Time	Value
03/04/2024 0826	0.89

AST, Plasma (U/L)

Date/Time	Value
03/04/2024 0826	19

CA 125 (U/mL)

Date/Time	Value
11/22/2024 1344	94.80 (H)
05/17/2024 0824	10.20
04/19/2024 0950	10.90
03/29/2024 0948	11.00
03/04/2024 0826	10.30
02/09/2024 0919	12.30
01/19/2024 0925	13.50

Case Report

Surgical Pathology Report

Case: SX23-11637

Authorizing Provider: Cottrill, Hope M., MD Collected:
12/14/2023 01:41 PM

Ordering Location: BAPTIST HEALTH LEXINGTON Received:
12/14/2023 01:56 PM

OR

Pathologist: Crowe, Patrick C., MD

Specimens: 1) - Large Intestine, Sigmoid Colon, COLON NODULES

2) - Sentinel Lymph Node, LEFT PELVIC SENTINEL NODE

3) - Large Intestine, Sigmoid Colon, COLON BIOPSY

4) - Uterus with Cervix, Bilateral Tubes and Ovaries

Clinical

Information

Endometrial cancer

Final Diagnosis

1. SPECIMEN SUBMITTED AS "COLON NODULES":

Desmoplastic stroma involved by carcinoma with frequent calcification consistent with serous carcinoma.

2. LYMPH NODE, LEFT PELVIC, SENTINEL NODE EXCISION:

1 lymph node negative for metastatic carcinoma (0/1).

3. SPECIMEN SUBMITTED AS "COLON BIOPSY":

Peritoneal surface involved by serous carcinoma.

4. UTERUS, CERVIX, BILATERAL OVARIES AND FALLOPIAN TUBES, HYSTERECTOMY AND BILATERAL SALPINGO-OOPHORECTOMY:

Serous carcinoma involving primarily an endometrial polyp with myometrial invasion.

Tumor present in the left fallopian tube.

Extensive adenomyosis.

Leiomyomata.

No involvement of cervical stroma or bilateral ovaries.

See comment and tumor synoptic for additional details.

UTERUS ENDOMETRIUM TEMPLATE:

SPECIMEN TYPE/ PROCEDURE: Hysterectomy and bilateral salpingo-oophorectomy

LYMPH NODE SAMPLING: Left pelvic sentinel node only

SPECIMEN INTEGRITY: Intact

TUMOR SIZE (greatest dimension): 2.5 x 2.5 x 1.9 cm

HISTOLOGIC TYPE: Serous carcinoma

HISTOLOGIC GRADE: N/A

MYOMETRIAL INVASION (Present/Not identified): Present

DEPTH OF INVASION (mm): 8

MYOMETRIAL THICKNESS (mm): 15

PERCENTAGE OF MYOMETRIAL INVASION: 53%

INVOLVEMENT OF CERVICAL STROMA: Not identified

EXTENT OF INVOLVEMENT OF OTHER TISSUE/ORGANS: Tumor present in the left fallopian tube and colonic serosa

PERITONEAL/ASCITIC FLUID CYTOLOGY: Malignant cells present

PELVIC LYMPH NODES (SUBMITTED/NONE): Left pelvic sentinel node only

NUMBER OF PELVIC LYMPH NODES EXAMINED: 1

NUMBER OF PELVIC SENTINEL NODES EXAMINED: 1

LATERALITY OF PELVIC LYMPH NODES EXAMINED: Left

NUMBER OF PELVIC LYMPH NODES WITH MACROMETASTASIS: 0

NUMBER OF PELVIC LYMPH NODES WITH MICROMETASTASIS: 0

NUMBER OF PELVIC LYMPH NODES WITH ITC'S: 0

LYMPHVASCULAR INVASION: Not identified

MSI TESTING: Intact expression per previous biopsy

ADDITIONAL PATHOLOGIC FINDINGS: Adenomyosis, leiomyoma

ANCILLARY STUDIES: HER2/neu and Napsin immunohistochemical staining

AJCC PATHOLOGIC STAGE: (COMPLETED BY PATHOLOGIST, BASED ONLY ON TISSUE FINDINGS, MORE EXTENSIVE DISEASE MAY NOT BE KNOWN TO THE PATHOLOGIST)

pT= 3b

pN= 0(sn)

Electronically signed by Crowe, Patrick C., MD on 12/19/2023 at 4:56 PM

Comment

Tumor within the endometrium is primarily within the polyp but does show myometrial invasion greater than 50%. There is tumor present within the lumen of the left fallopian tube. Due to concern for the possibility of a clear cell component immunohistochemical stain for Napsin was undertaken with appropriate controls. No Napsin positivity is seen in the tumor. HER2/neu stain was also performed and was negative (0+). Please see tumor template for additional details.

Her2/neu: negative

PDL score 0

CT CHEST W IV CONTRAST authorized by: Tricia I Fredericks, MD

Study Result

Narrative & Impression

CLINICAL INDICATION:

Endometrial Cancer, restaging

TECHNIQUE:

Multiple axial CT images were obtained from thoracic inlet through pubic symphysis following administration of IV contrast, Omnipaque 300, 100 mL. Reformatted images of the abdomen and pelvis in the coronal and sagittal planes were generated from the axial data set to facilitate diagnostic accuracy.

Total DLP (Dose-Length Product): 900.61 mGy.cm. Please note: The reported value represents the total of one or more individual components during the CT acquisition on this date and at this time, and as such, the same value may appear in more than one CT report depending on the interpreting/reporting physicians.

COMPARISON:

None.

FINDINGS:

Chest:

Lymph Nodes and Mediastinum: No lymphadenopathy by CT size criteria. No mediastinal mass lesions. No suspicious thyroid findings.

Cardiovascular: The heart is normal in caliber. Thoracic great vessels are patent.

Lungs and Pleura: No suspicious lung nodules to suggest metastatic disease. No pleural effusions or suspicious thickening.

Musculoskeletal and Body Wall: No clearly aggressive bone lesions.

Abdomen/Pelvis:

Solid Abdominal Organs: There are scattered low-attenuation nodules within the liver which are all subcentimeter and too small to characterize. There are unchanged compared to November 2023 and are likely benign. The gallbladder is normal. The bile duct is upper limits of normal. No pancreatic duct dilation, nodule, or mass. The spleen and adrenal glands are normal. The kidneys are normal.

GI Tract/Mesentery/Peritoneum: There is a moderate-sized paraesophageal hernia. No bowel obstruction. The appendix is normal. There is colonic diverticulosis. There is soft tissue thickening along the left paracolic gutter extending into the pelvis which appears to partially tethered the distal descending colon this is new from the prior examination. (Series 3 image

188 to image 213).

There is infiltration of fat in the left upper quadrant which appears new from the prior examination measuring 15 mm. Other subsolid nodules are also noted in this region (series 3 image 72, image 78).

Pelvic Viscera: The bladder is normal. The uterus is absent. There is a fluid collection in the pelvis measuring up to 3.1 cm. Soft tissue thickening is noted along the left pelvic sidewall (series 3 image 208), new from the prior examination.

Lymph Nodes/Vasculature: No lymphadenopathy is evident. No large vessel occlusion.

Free Fluid: No free fluid

Musculoskeletal and Body Wall: Fat-containing periumbilical no suspicious osseous lytic or blastic lesion.

IMPRESSION:

Chest: No evidence of metastatic disease in the chest

Abdomen/Pelvis:

New peritoneal thickening along the left paracolic gutter and left pelvic sidewall. Some of the thickening has increased density. There is also a loculated fluid collection in the pelvis. These findings are indeterminate and could represent recurrent malignancy with peritoneal spread. Differential diagnosis would include the sequela of perforated diverticulitis. Correlate with history of recent abdominal and pelvic pain and/or symptoms of diverticulitis.

Subtle increased low density nodularity in the fat of the left upper quadrant. This is nonspecific but could represent peritoneal disease.

Assessment/Plan

Pap Smear was not collected today.

Problem 1: Stage IIIB serous endometrial cancer, s/p resection - encounter for antineoplastic chemotherapy

Assessment and plan 1: Per previous note: I recommend adjuvant treatment with carbo/taxol x 6. I

do not recommend immunotherapy as not recommended by NCCN for a Stage IIIB with no measurable disease. Her2/neu negative so no role for trastuzumab.

PLD1-, MSI testing intact, p53negative, ER/PR+

S/p VBT completed 8/14/24

Imaging and symptoms could represent early recurrence. We will get a CA125, PET scan and send her tumor for CARIS testing.

PET scan consistent with small volume recurrence at the cuff and some suggestion of carcinomatosis in left lateral abdomen. Reviewed that at this point, not likely to be curable. I would recommend treatment with doxil/bev. I discussed expected side effects and schedule of treatment. Another option would be lenvatanib/pembro but tends to be toxic and poorly tolerated. Shared discussed with my partners who would recommend doxil/bev. Can wait to start after Christmas, I don't think it would make a big difference.

She is understandably tearful. She would like some time to make the decision. She is also considering a second opinion, recommend MD Anderson as they also have a lot of clinic trials. She wants to think and discuss with her PCP who she trusts.

Her CARIS is pending. The only trial we would have available would be through comboMATCH if she has a targetable mutation.

She will call with her decision.

Problem 2: Obesity Body mass index is 32.7 kg/m².

Assessment and plan 2: complicates all aspects of care

Problem 3: Anxiety

Assessment and plan 3: treated per PCP with ativan, celexa. I encourage counseling to help with anxiety affecting quality of life. SW to see today and help set up with resources

Problem 4: Family history of endometrial cancer

Assessment and plan 4: Genetic counseling referral placed

Problem 6: hip and knee pain

Assessment and plan 6: Continue tramadol PRN

45 minutes was spent on this encounter; including preparing to see the patient, which involved review/interpretation of diagnostics and reports; obtaining and/or reviewing separately obtained history; performing appropriate physical exam; ordering/scheduling medications, tests or procedures; communicating findings and counseling/educating the patient, family and/or caregiver; documentation in EMR; and care coordination. Team based care includes nurse intake, review of prior documentation, history, physical exam, discussion of plan of care, appointment scheduling, lab orders, phlebotomy, and documentation.

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LEXINGTON KY 40536-0001
Dept: 859-323-5553
Dept Fax: 859-257-7146
Loc: 859-323-5000

Additional Documentation

Vitals: BP **147/86 !** (Abnormal) Pulse 92 Temp **35.9 °C (96.6 °F) !** (Abnormal) Resp 14
Wt 81.1 kg (178 lb 12.7 oz) LMP 12/01/2010 BMI 32.70 kg/m² BSA 1.88 m² Pain Sc 0-No pain

Flowsheets: Patient-Reported Data, PHQ-9 (Adult), Markey CARES Tobacco Treatment Program,
Learning Needs Screening

SmartForms: AMB SF STEADI FALL RISK

Communications

✉ Letter sent to Charles S Giles, MD

📎 Provider Notes
Sent 12/5/2024

Media

From this encounter

Surprise Billing Notification - Electronic signature on 11/27/2024 2:17 PM - E-signed

🗣️ Welcome Accident Related Questionnaire

Question	11/27/2024 2:16 PM EST - Filed by Patient
Is this visit related to a workers' compensation, auto, or Black Lung?	No

🗣️ Nccn Distress Questionnaire

Question	12/3/2024 6:27 PM EST - Filed by Patient
Appointment Type	Other
Based on the past week, please select the number that best describes how much distress you have had. 0 being none and 10 being the most extreme.	6
Physical Concerns: Are you having difficulty with the following? Select: Yes or No	
Pain	Yes
Sleep	No
Fatigue	Yes
Tobacco use	No
Substance use	No
Memory/Concentration	No
Sexual health	No
Changes in eating	No
Loss or change of physical abilities	No

12/3/2024 6:27 PM EST -

Filed by Patient

Question

Emotional concerns: Are you having difficulty with the following? Select Yes or No

Worry or anxiety	Yes
Sadness or Depression	No
Loss of interest or enjoyment	No
Grief or loss	No
Fear	Yes
Loneliness	No
Anger	No
Changes in appearance	No
Feelings of worthlessness or being a burden	No

Social Concerns: Are you having difficulties with the following? Select Yes or No

Relationship with spouse or partner	No
Relationship with Children	No
Relationships with family members	No
Relationship with friends or coworkers	No
Communication with health care team	No
Ability to have children	No

Practical concerns: Are you having difficulties with following? Select Yes or No

Taking care of myself	No
Taking care of others	No
Work	No
School	No
Housing	No
Finances	No
Insurance	No
Transportation	No
Child care	No
Having enough food	No
Access to medicine	No
Treatment Decisions	Yes

Spiritual/Religious Concerns? Select Yes or No

Sense of meaning or purpose	No
Change in faith or beliefs	No
Death, dying or afterlife	No
Conflict between beliefs and cancer treatments	No
Relationship with the sacred	No
Ritual or dietary needs	no

Other concerns

The unknown in regard to the cancer returning and how to treat it.

Distress Total (range: 0 - 10)	6
Physical Concerns Total (range: 0 - 12)	1
Emotional Concerns Total (range: 0 - 9)	0
Social Concerns (range: 0 - 6)	0
Practical Concerns Total (range: 0 - 12)	1
Spiritual/Religious Concerns Total (range: 0 - 6)	0
NCCN Total (range: 0 - 42)	2

Education

No education to display

Linked Episodes

Oncology Treatment Noted 1/2/2024

Orders Placed

None

Medication Changes

As of 12/5/2024 10:44 AM

	Refills	Start Date	End Date
Discontinued or Completed: acetaminophen (Tylenol) 325 MG tablet			
Patient-reported medication			
Discontinued or Completed: diclofenac (Voltaren) 1 % topical gel			
Discontinued or Completed: ibuprofen 600 MG tablet			
Patient-reported medication			
Discontinued or Completed: ondansetron ODT (Zofran-ODT) 8 MG disintegrating tablet			
Discontinued or Completed: prochlorperazine (Compazine) 10 MG tablet			

Medication List at End of Visit

As of 12/5/2024 10:44 AM

	Refills	Start Date	End Date
amLODIPine (Norvasc) 5 MG tablet	—	2/23/2024	—
Patient-reported medication			
citalopram (CeleXA) 10 MG tablet	—	2/23/2024	—
Patient-reported medication			
diphenhydrAMINE (Benadryl) 25 MG capsule	—		—
Take 1 capsule (25 mg) by mouth at night if needed. - Oral			
Patient-reported medication			
lisinopril 10 MG tablet	—	2/28/2024	—
Take 1 tablet (10 mg) by mouth 1 (one) time each day. - Oral			
Patient-reported medication			
LORazepam (Ativan) 1 MG tablet	—		—
Patient-reported medication			
traMADol (Ultram) 50 MG tablet	0	3/29/2024	—
Take 1 tablet (50 mg) by mouth every 8 (eight) hours if needed for severe pain. - Oral			

Visit Diagnoses

Primary: Endometrial cancer (CMS/HCC) C54.1
Class 1 obesity due to excess calories without serious comorbidity with body mass index (BMI) of 32.0 to 32.9 in adult E66.811, E66.09, Z68.32
Anxiety F41.9
Arthralgia, unspecified joint M25.50